

Patient ID : PA04

<i>CT Scan of Whole Abdomen</i>
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Report:

Plain (only oral contrast for bowel opacification given) axial scans were taken in the region of interest. Intravenous contrast-enhanced axial scans were taken from the domes of the diaphragm to the pubic symphysis. Study reveals:

Liver: - Shrunken (13 cm) and shows diffusely coarsened parenchymal echotexture with nodular surface and shows mildly atrophied right lobe with hypertrophied caudate lobe. Liver enhancement is normal. No evidence of intrahepatic biliary radical dilatation. No focal space-occupying lesion is seen.

Gall bladder: - Is normal in diameter and wall thickness. No hyperdense intraluminal calculus is seen. No focal mass lesion is seen.

CBD: - is normal in diameter, with normal walls. No hyperdense intraluminal calculus is seen. **Portal vein** is dilated (16 mm in diameter). **SP axis** is patent.

Pancreas: - Is normal in size, attenuation and enhancement. Pancreatic duct is not dilated. No evidence of pancreatic calcification. No evidence of any peripancreatic inflammation or fluid collection.

Spleen: - Is normal in size, attenuation and enhancement. No focal space-occupying lesion is seen.

Kidneys: - Both kidneys are normal in size, position and attenuation. Kidney enhancement is normal and corticomedullary differentiation is maintained. No evidence of hydronephrosis, no calculi seen.

Bilateral **adrenal glands** are normal in size and attenuation.

Urinary Bladder: - Is partially distended, normal in position. Bladder walls are not thickened, no mass lesion is seen. Foleys catheter bulb is seen in situ. No intraluminal calculus is seen.

Uterus: Atrophied. No focal mass lesion is seen.

Bilateral adnexal region is seen normally. No focal mass lesion is seen.

Aorta and IVC are normal. No evidence of retroperitoneal or mesenteric lymphadenopathy. Mild ascites is seen.

No abnormal bowel wall thickening or mass lesion is seen. No abnormally dilated bowel loops is seen.

Impression: Cirrhosis liver with dilated portal vein and mild ascites.

Advise: Clinical Correlation